OPERATIONAL EXCELLENCE · WHAT THE RESEARCH SAYS

Staff fix a problem on their own. The next day, it is back.

In 93% of cases, no one else ever heard about it.

A field study in U.S. hospitals found this pattern. More than 20 years later, a new study found nearly the same thing.

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THE STUDY

What did the researchers see?

Anita Tucker and Amy Edmondson spent 239 hours observing 26 nurses in nine U.S. hospitals, most of them known for strong nursing care.

They recorded 194 breakdowns in everyday work. Most were familiar: a medication that never arrived, a broken device, supplies that ran out, or a regular meal tray sent to a diabetic patient.

None of these problems were complex, and none needed rare expertise to fix.

AN EXAMPLE FROM THE STUDY

One fix ends the moment. The other ends the problem.

On a night shift, clean linen ran out. The nurse took some from another unit and finished her work. The moment passed, but nothing stopped the same thing from happening the next night.

The other option is to finish the work, then tell the linen department or a manager so the cause gets fixed.

The study calls the first first-order problem solving and the second second-order problem solving.

FINDINGS

Most problems were patched on the spot, then forgotten

93%

of problems: staff did just enough to finish the task and told no one.

7%

of responses alerted someone who could fix the cause, even with a generous definition.

33

minutes of each eight-hour shift, on average, went to coping with failures that could have been removed.

IT WASN'T A LACK OF EFFORT

The nurses were giving more than was asked of them

Nine of the 10 nurses observed for a full shift stayed an average of 45 minutes after it ended, without extra pay, to finish caring for their patients.

Seven of the nine nurses interviewed said it felt good to find a way around an obstacle.

That is the paradox: the better people handle a problem alone, the more it stays hidden from those who could fix it.

MORE THAN 20 YEARS LATER

Staff saw the problem, but mostly did not report it

96.2%

of built-in failures in a simulation were noticed by 80 nurses in a Belgian study published in 2026.

21.9%

of cases led to action on the cause, such as formal reporting or escalation.

A moving patient-safety story stirred strong feelings, but it did not increase reporting.

WHAT THE TWO STUDIES RECOMMEND

What can leaders do?

1

Be present where the work happens during every shift, even briefly, so small problems reach you before they pile up.

2

Make reporting a problem safe, never a sign that someone can't do the job.

3

Answer every report with an action the team can see, and hold a short debrief after work: what got in our way today?

> **How many problems did your team fix on its own today that will be back tomorrow?**

SOURCES AND LIMITS

What this evidence can and cannot tell us

The 2003 study is qualitative with a small sample. Its figures cover only what was observed.

The 2026 study took place in a simulation, not on a real ward, with 80 participants.

Neither study included Arab hospitals. Applying the idea to other sectors is an inference.

Editorial disclaimer: The views and analyses presented in this article represent the author's professional interpretation based on the cited sources and do not necessarily represent the official positions of the organizations mentioned. For the original, complete and most current information, please consult the primary sources and official documents listed in the references.

[1] Tucker, A. L., & Edmondson, A. C. (2003). Why Hospitals Don't Learn from Failures. *California Management Review*, 45(2), 55–72.

[2] Vanbelleghem, S., De Regge, M., Van Nieuwenhove, Y., & Gemmel, P. (2026). Embedding Sustainable Second-Order Problem-Solving Behaviour in Hospital Nurses via Simulation-Based Education. *Journal of Advanced Nursing*.

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